Discharge Summary:

Patient Name: [Insert Name]
Medical Record Number: [Insert Number]
Date of Admission: [Insert Date]
Date of Discharge: [Insert Date]
Admitting Diagnosis: ST-elevation myocardial infarction (STEMI) and right-sided pneumonia
Principal Diagnosis at Discharge: Takotsubo cardiomyopathy and nonobstructive coronary artery disease

Hospital Course:
The patient, a 75-year-old male with a past medical history significant for polycythemia vera, obstructive sleep apnea, hypertension, hyperlipidemia, aortic stenosis, anemia, chronic kidney disease stage IV, and stroke, was admitted to the hospital with ST-elevation myocardial infarction (STEMI) and right-sided pneumonia. Upon arrival, the patient was intubated and history was obtained from the patient's family. The patient was maintained on dual antiplatelet therapy and metoprolol tartrate 50 mg twice daily. The patient's hospital stay was uneventful with the patient receiving IV furosemide and was extubated a week later. Cardiac catheterization revealed nonobstructive coronary artery disease. The patient was discharged with hydroxyurea 1000 mg and ruxolitinib 5 mg both twice daily.

Hospitalization:
The patient was transferred to our facility for further management of a ST-elevation myocardial infarction (STEMI). He had presented 3 days earlier to an outside hospital with shortness of breath and productive cough. On day 3 of admission, he developed worsening shortness of breath with tachypnea and oxygen desaturation. His electrocardiogram (EKG) showed ST-segment elevation in anterolateral precordial leads with QT/QTc interval prolongation and troponin T then was slightly elevated. Chest X-ray (CXR) showed diffuse pulmonary edema. He was started on intravenous (IV) furosemide and intubated for hypoxic respiratory failure.

Past Medical History:
The patient's past medical history was significant for polycythemia vera, obstructive sleep apnea, hypertension, hyperlipidemia, aortic stenosis, anemia, chronic kidney disease stage IV, and stroke. He had undergone cardiac catheterization 2 years ago which had shown mild to moderate stenosis of the ostial left anterior descending (LAD) artery and first diagonal branch.

Laboratory Data:
The patient's platelet count was elevated to 3321000/µL and later peaked at 4145000/µL. His white cell count was elevated to 13100/µL, hemoglobin was 7.5 g/dL and creatinine was elevated.

Diagnostic Imaging:
Transthoracic echocardiography showed moderately decreased left ventricular systolic function with dyskinetic to akinetic distal half of both ventricles consistent with takotsubo cardiomyopathy.

Medications:
The patient was maintained on dual antiplatelet therapy and metoprolol tartrate 50 mg twice daily and was started on heparin infusion. After cardiac catheterization, the patient was discharged with hydroxyurea 1000 mg and ruxolitinib 5 mg both twice daily.

Follow-Up Care:
The patient was advised to follow up with primary care physician and cardiology on a regular basis. The patient was advised to continue follow up care for polycythemia vera and hypertension. The patient was

also advised to maintain a diet low in sodium and to exercise regularly.

Attending Physician:
[Insert Name], MD, [Insert Title]